ISSUED, VALID FROM 30 SEPTEMBER 2026

Minor Wound Care

Patient Group Direction, version 011, issued 30 September 2026. Two arms: co-amoxiclav and flucloxacillin

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Scope of this PGD

This PGD authorises the supply of an oral antibiotic for an infected minor wound in patients aged 2 years and over, alongside wound care advice and an assessment of tetanus status.

- **Co-amoxiclav is used for infected bite wounds and heavily contaminated wounds. Flucloxacillin is used for infected non-bite wounds. The choice is made on the mechanism of the wound.**
- **Every observation in Appendix 1 must be measured and recorded before any supply. If any threshold is breached, refer.**
- **Tetanus immunisation status must be established and recorded for every patient. See Appendix 1.**
- Both arms are beta-lactams. A penicillin-allergic patient cannot be treated under this PGD and is referred, with the likely alternative named.

Arm 1 , Co-amoxiclav (bites and heavily contaminated wounds)

Summary of NICE guidance for cellulitis and infected wounds

Cellulitis and erysipelas: antimicrobial prescribing, NICE guideline NG141, published 27 September 2019; minor changes November 2019 (committee discussion on co-amoxiclav in children) and January 2022 (wording on macrolides in pregnancy). Summarised 9 September 2026.

What the guideline covers

NG141 sets out an antimicrobial prescribing strategy for adults, young people, children and babies aged 72 hours and over with cellulitis and erysipelas.

NICE describes cellulitis and erysipelas as infections of the subcutaneous tissues that usually result from contamination of a break in the skin, characterised by acute localised inflammation and oedema, with erysipelas lesions more superficial and having a well-defined raised margin.

Assessment and differential diagnosis

To ensure cellulitis and erysipelas are treated appropriately, exclude other causes of skin redness such as an inflammatory reaction to an immunisation or an insect bite, or a non-infectious cause such as chronic venous insufficiency.

Other possible diagnoses NICE lists at reassessment are gout, superficial thrombophlebitis, eczema, allergic dermatitis and deep vein thrombosis.

Before treating, consider drawing around the extent of the infection with a single-use surgical marker pen to monitor progress, and be aware that redness may be less visible on darker skin tones.

Consider a swab for microbiological testing only if the skin is broken and there is a penetrating injury, exposure to water-borne organisms, or the infection was acquired outside the UK.

Features that suggest a more serious illness or condition are lymphangitis, orbital cellulitis, osteomyelitis, septic arthritis, necrotising fasciitis and sepsis.

Infection around the eyes or nose (the triangle from the bridge of the nose to the corners of the mouth, or immediately around the eyes including periorbital cellulitis) is of more concern because of the risk of a serious intracranial complication.

Manage any underlying condition that may predispose to cellulitis, for example diabetes, venous insufficiency, eczema, or oedema (which may be an adverse effect of medicines such as calcium channel blockers).

When antibiotics are indicated

Offer an antibiotic to people with cellulitis or erysipelas.

When choosing the antibiotic, take account of the severity of symptoms, the site of infection (for example near the eyes or nose), the risk of uncommon pathogens (penetrating injury, water-borne exposure, infection acquired outside the UK), previous swab results, and the person's MRSA status if known.

Give oral antibiotics first line if the person can take oral medicines and the severity of their condition does not require intravenous antibiotics.

Do not routinely offer antibiotic prophylaxis to prevent recurrent cellulitis or erysipelas.

First choice flucloxacillin: doses and course length

For adults aged 18 years and over, the first-choice antibiotic is oral flucloxacillin 500 mg to 1 g four times a day for 5 to 7 days; NICE noted that in September 2019, 1 g orally four times a day was off label.

For children aged 1 month and over, oral flucloxacillin is 62.5 mg to 125 mg four times a day at 1 month to 1 year, 125 mg to 250 mg four times a day at 2 to 9 years, and 250 mg to 500 mg four times a day at 10 to 17 years, for 5 to 7 days.

Antibiotic choice for children under 1 month should be based on specialist advice.

A longer course, up to 14 days in total, may be needed based on clinical assessment, but skin takes some time to return to normal and full resolution of symptoms at 5 to 7 days is not expected.

If flucloxacillin oral solution is not tolerated because of poor palatability, consider capsules.

The age bands apply to children of average size; the prescriber should use them alongside the severity of the condition and the child's size relative to the average for their age.

Alternatives in penicillin allergy or where flucloxacillin is unsuitable

For adults, the alternatives are clarithromycin 500 mg twice a day for 5 to 7 days, erythromycin (in pregnancy) 500 mg four times a day for 5 to 7 days, or doxycycline 200 mg on the first day then 100 mg once a day for 5 to 7 days in total. None of these alternatives is authorised under this PGD: refer and name the likely alternative.

For children, alternatives are clarithromycin (1 month to 11 years, weight-based: under 8 kg 7.5 mg/kg twice a day, 8 to 11 kg 62.5 mg twice a day, 12 to 19 kg 125 mg twice a day, 20 to 29 kg 187.5 mg twice a day, 30 to 40 kg 250 mg twice a day; 12 to 17 years 250 mg to 500 mg twice a day) for 5 to 7 days, erythromycin in pregnancy at 8 to 17 years 250 mg to 500 mg four times a day for 5 to 7 days, or co-amoxiclav if flucloxacillin is unsuitable but there is no penicillin allergy.

NICE states erythromycin is preferred if a macrolide is needed in pregnancy, for example with true penicillin allergy where the benefits of treatment outweigh the harms.

If infection is near the eyes or nose, the first choice is co-amoxiclav 500/125 mg three times a day for 7 days, with clarithromycin plus metronidazole as the alternative in penicillin allergy, and NICE advises considering specialist advice.

If MRSA is suspected or confirmed, other antibiotics may be appropriate based on microbiological results and specialist advice.

See the BNF or BNF for Children for dosing in hepatic or renal impairment, pregnancy and breastfeeding.

Referral and seeking specialist advice

Refer people to hospital if they have any symptoms or signs suggesting a more serious illness or condition, such as orbital cellulitis, osteomyelitis, septic arthritis, necrotising fasciitis or sepsis.
Consider referral to hospital, or seek specialist advice, if the person is severely unwell, has infection near the eyes or nose (including periorbital cellulitis), could have uncommon pathogens (penetrating injury, water-borne exposure, infection acquired outside the UK), has spreading infection not responding to oral antibiotics, has lymphangitis, or cannot take oral antibiotics.

Advice to the patient and reassessment

When prescribing antibiotics, give advice about possible adverse effects, that the skin will take some time to return to normal after the course has finished, and to seek medical help if symptoms worsen rapidly or significantly at any time or do not start to improve within 2 to 3 days.
Reassess if symptoms worsen rapidly or significantly at any time, do not start to improve within 2 to 3 days, or the person becomes systemically very unwell, has severe pain out of proportion to the infection, or has redness or swelling spreading beyond the initial presentation (allowing for some initial spread, and for redness being less visible on darker skin).
At reassessment, take account of other possible diagnoses, any predisposing condition, any features of a more serious illness, any microbiology results, and previous antibiotic use which may have led to resistant bacteria.
Consider taking a swab at reassessment if the skin is broken and this has not already been done; if a swab has been sent, review the antibiotic when results are available and change it if the infection is not improving, using a narrow-spectrum antibiotic if possible.

Patient Group Direction for the supply of co-amoxiclav 500/125mg tablets for infected bite wounds and heavily contaminated wounds in patients aged 12 years and over.

Choose this arm where the wound is a bite, or is heavily contaminated with soil or organic material. Pasteurella and Eikenella are not reliably covered by flucloxacillin. Arm 1 applies only where tetanus management for this wound, including any human tetanus immunoglobulin (HTIG), has been completed and is recorded, or the wound is not tetanus-prone; otherwise refer. A heavily contaminated wound is tetanus-prone and usually high-risk, so HTIG will often have been needed: it is not supplied under this or any GRH PGD.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. Neither medicine is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to wound assessment, documented and overseen by the Get Real Health team.
- Must be competent in measuring and recording the observations in Appendix 1 for the age of the patient, including capillary refill in children, and in recognising the necrotising fasciitis features listed there.
- Must be able to assess tetanus immunisation status and apply the referral rule below.
- Must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Infected animal or human bite wounds, and infected heavily contaminated wounds, in patients aged 12 years and over, where tetanus management for the wound, including any HTIG, has been completed and recorded, or the wound is not tetanus-prone.
Inclusion criteria	• Aged 12 years and over. • A bite wound, or a heavily contaminated wound, with signs of infection: erythema, warmth, swelling, tenderness or purulent discharge. • **The bite is NOT to the face or hand.** • **All observations within the thresholds in Appendix 1.** • Tetanus status resolved, and recorded, as ONE of: (a) the wound is not tetanus-prone (UKHSA definition, Appendix 1); or (b) tetanus management for this wound, including any human tetanus immunoglobulin (HTIG) and any vaccine dose, has been completed and is recorded (for example HTIG and Td/IPV given at an emergency department at the time of injury, or a Td/IPV dose given today under the Tetanus (Td/IPV) PGD, or immunisation up to date with nothing further indicated). Where HTIG or a vaccine dose is indicated and has not been given, refer; do not supply under this arm. • Valid informed consent given. Where the patient is under 16, consent from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis recorded. • No exclusion criterion present.
Exclusion criteria	**Refer, do not supply, where any of the following is present.**

	● **ANY feature suggesting necrotising fasciitis: pain out of proportion to appearance, rapidly advancing erythema, crepitus, skin necrosis, bullae or dusky discoloration. Emergency assessment.** ● **Any observation outside the thresholds in Appendix 1, or any sign of systemic illness or sepsis.** ● **HIGH-RISK tetanus-prone wound, where human tetanus immunoglobulin (HTIG) may be indicated: a tetanus-prone wound with heavy contamination by material likely to contain tetanus spores (for example soil or manure) and/or extensive devitalised tissue (UKHSA definition), UNLESS tetanus management for this wound, including HTIG, has already been completed and is recorded (for example at an emergency department at the time of injury). Where it has not, refer the same day; HTIG is not covered by this PGD. Presentation more than 6 hours after injury, a puncture wound or a burn make a wound tetanus-prone, not high-risk: a wound that needs only a vaccine dose is handled under the Tetanus (Td/IPV) PGD and does not exclude this supply. Burns and wounds with systemic sepsis are outside this PGD in any case: refer.** ● Wound requiring closure, exploration or surgical review, or a retained foreign body. ● Deep or penetrating wound, or any wound over a joint, tendon or bone. ● Bite to the face or hand. ● Abscess requiring drainage. ● Immunosuppression of any kind, or diabetic foot wound. ● An antibiotic already taken for this episode. ● Known penicillin or cephalosporin allergy. Refer; see the note on alternatives below. ● History of co-amoxiclav-associated jaundice or hepatic dysfunction. ● Infectious mononucleosis or acute lymphoblastic leukaemia. ● Severe hepatic dysfunction, or eGFR below 30. ● Under 12 years of age.
Cautions	● Co-amoxiclav may be supplied in pregnancy and breastfeeding where clinically indicated. ● Possible increased anticoagulant effect; where the patient is anticoagulated, refer rather than supply. ● Advise that any rash should stop the course and prompt review.
Actions if the patient is excluded or declines	Explain why treatment cannot be supplied and arrange the appropriate assessment. Where any necrotising fasciitis feature is present, arrange EMERGENCY assessment. **FOR PENICILLIN ALLERGY: both arms of this PGD are beta-lactams, so a penicillin-allergic patient cannot be treated here. Refer to the GP and tell them what is likely to be needed, rather than saying only that alternatives exist. For an infected bite the usual choice is doxycycline with metronidazole; for a non-bite wound infection, clarithromycin or doxycycline. Name this in the referral so the patient is not sent away empty-handed.** **FOR TETANUS: where the wound is high risk and immunoglobulin may be indicated, refer the same day; immunoglobulin is not covered by this or any GRH PGD. Where a vaccine dose is all that is needed (last dose more than 10 years ago whatever the dose count, or an incomplete history) and the patient is aged 10 or over, the Tetanus (Td/IPV) PGD may be used. Arm 1 (bites and heavily contaminated wounds) applies only**

	where tetanus management for the wound, including any HTIG, has been completed and recorded, or the wound is not tetanus-prone: a patient referred for HTIG may return for the antibiotic once that management is complete and recorded. Document the advice and the decision, and inform the GP where the reason for exclusion is a new clinical finding.

The medicine

Name, form and strength	Co-amoxiclav 500/125mg tablets.
Legal category	POM.
Route and method	Oral. Take at the start of a meal to reduce gastrointestinal upset.
Dose and frequency	One tablet (500/125mg) three times a day for 5 to 7 days.
Quantity to be supplied	**15 tablets for a 5 day course, or 21 tablets for a 7 day course.**
Maximum treatment period	7 days. **One course per episode. A second course is not authorised; refer.**
Adverse effects	Very common: diarrhoea, nausea. Common: rash, vomiting, candidiasis. Uncommon: abdominal pain, indigestion, headache. Rare but serious: anaphylaxis, Stevens-Johnson syndrome, toxic epidermal necrolysis, cholestatic hepatitis, C. difficile infection.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	● That valid informed consent was given. ● Patient name, address, date of birth, and the GP with whom they are registered. ● The observations from Appendix 1. ● The mechanism, site and extent of the wound, and whether it was a bite. ● Tetanus immunisation status and the action taken, including that tetanus management for this wound (any HTIG and any vaccine dose) was completed and where and when, or that the wound is not tetanus-prone. ● Which antibiotic was chosen and why. ● Name and registration number of the healthcare professional supplying. ● Name of the medicine, date of supply, dose, form, route and quantity supplied. ● Batch number and expiry date. ● Advice given, including advice given if excluded or declining treatment. ● Details of any adverse drug reactions and the actions taken. ● That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults: 8 years. Under 18s: until the 25th birthday, or the 26th birthday where the patient was 17 when treatment finished.
Storage	Store below 25C.

Information for the patient

Written information	Supply the co-amoxiclav patient information leaflet.
Counselling	• Take one tablet three times a day at the start of a meal, and finish the course. • **Seek help THE SAME DAY if the pain becomes severe or out of proportion to how the wound looks, if redness spreads quickly, if the skin darkens or blisters, or if you develop fever, shivering or feel generally unwell.** • **Seek help if you see red streaks tracking away from the wound.** • Stop and seek urgent help if you develop a rash, wheeze, or swelling of the lips or tongue. • Report yellowing of the eyes or skin, or dark urine, even after finishing. • Keep the wound clean and dry. Come back if it is no better in 2 to 3 days.
Follow-up	Review in 2 to 3 days if not improving, and sooner if any warning sign develops. Ensure the tetanus question has been answered before the patient leaves.

Arm 2 , Flucloxacillin (non-bite wound infection)

Patient Group Direction for the supply of flucloxacillin for infected non-bite wounds and localised skin infection in patients aged 2 years and over.

Choose this arm for non-bite wound infection where staphylococcal or streptococcal infection is likely. Do NOT use it for bite wounds; use Arm 1.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. Neither medicine is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to wound assessment, documented and overseen by the Get Real Health team.
- Must be competent in measuring and recording the observations in Appendix 1 for the age of the patient, including capillary refill in children, and in recognising the necrotising fasciitis features listed there.
- Must be able to assess tetanus immunisation status and apply the referral rule below.
- Must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Infected non-bite wounds and localised skin and soft tissue infection in patients aged 2 years and over.
Inclusion criteria	• Aged 2 years and over. • **A NON-BITE wound with signs of infection.** • **All observations within the thresholds in Appendix 1.** • Tetanus immunisation status established and up to date, or the patient referred for it separately. • Valid informed consent given. Where the patient is under 16, consent from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis recorded. • No exclusion criterion present.
Exclusion criteria	**Refer, do not supply, where any of the following is present.** • **ANY feature suggesting necrotising fasciitis: pain out of proportion to appearance, rapidly advancing erythema, crepitus, skin necrosis, bullae or dusky discoloration. Emergency assessment.** • **Any observation outside the thresholds in Appendix 1, or any sign of systemic illness or sepsis.** • **HIGH-RISK tetanus-prone wound, where human tetanus immunoglobulin (HTIG) may be indicated: a tetanus-prone wound with heavy contamination by material likely to contain tetanus spores (for example soil or manure) and/or extensive devitalised tissue (UKHSA definition). Refer the same day; HTIG is not covered by this PGD. Presentation more than 6 hours after injury,**

	a puncture wound or a burn make a wound tetanus-prone, not high-risk: a wound that needs only a vaccine dose is handled under the Tetanus (Td/IPV) PGD and does not exclude this supply. Burns and wounds with systemic sepsis are outside this PGD in any case: refer. ● Wound requiring closure, exploration or surgical review, or a retained foreign body. ● Deep or penetrating wound, or any wound over a joint, tendon or bone. ● Bite to the face or hand. ● Abscess requiring drainage. ● Immunosuppression of any kind, or diabetic foot wound. ● An antibiotic already taken for this episode. ● **Any bite wound, animal or human. Flucloxacillin does not reliably cover Pasteurella multocida or Eikenella; use Arm 1.** ● Known penicillin or beta-lactam allergy. Refer; see the note on alternatives in Arm 1. ● History of flucloxacillin-associated jaundice or hepatic dysfunction. ● Under 2 years of age. ● Severe renal impairment (creatinine clearance below 10 mL/min).
Cautions	● **Flucloxacillin may be supplied in pregnancy and breastfeeding where clinically indicated. This aligns with the Skin and Soft Tissue Infection PGD.** ● Hepatic reactions may occur up to two months after treatment; advise the patient to report jaundice or dark urine.
Actions if the patient is excluded or declines	As Arm 1, including the named alternatives for penicillin allergy and the tetanus referral route.

The medicine

Name, form and strength	Flucloxacillin 500mg capsules; flucloxacillin 250mg/5mL oral suspension for children unable to swallow capsules.
Legal category	POM.
Route and method	Oral. Take on an empty stomach, one hour before or two hours after food.
Dose and frequency	Adults and children 10 years and over: 500mg four times daily. A child aged 10 to 17 who cannot swallow capsules takes the same 500mg dose as 10 mL of the 250mg/5mL suspension four times daily. CHECK THE VOLUME AGAINST THE STRENGTH BEFORE SUPPLY: the 250mg/5mL suspension delivers 50mg per mL. Children 2 to 9 years: 250mg four times daily, which is 5 mL of the 250mg/5mL suspension four times daily. CHECK THE VOLUME AGAINST THE STRENGTH BEFORE SUPPLY: the 250mg/5mL suspension delivers 50mg per mL. Duration 5 to 7 days.
Quantity to be supplied	Capsules: 20 for a 5 day course, 28 for a 7 day course. Suspension, 2 to 9 years (250mg, 5 mL four times daily): 100mL for 5 days, 140mL for 7 days. Suspension, 10 to 17 years unable to swallow capsules (500mg, 10 mL four times daily): 200mL for 5 days, 280mL for 7 days.
Maximum treatment	7 days.

period	**One course per episode. A second course is not authorised; refer.**
Adverse effects	Very common: nausea, diarrhoea. Common: rash, vomiting, abdominal discomfort. Rare but serious: anaphylaxis, cholestatic jaundice and hepatitis (may be delayed up to two months), C. difficile infection.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	● That valid informed consent was given. ● Patient name, address, date of birth, and the GP with whom they are registered. ● The observations from Appendix 1. ● The mechanism, site and extent of the wound, and whether it was a bite. ● Tetanus immunisation status and the action taken. ● Which antibiotic was chosen and why. ● Name and registration number of the healthcare professional supplying. ● Name of the medicine, date of supply, dose, form, route and quantity supplied. ● Batch number and expiry date. ● Advice given, including advice given if excluded or declining treatment. ● Details of any adverse drug reactions and the actions taken. ● That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults: 8 years. Under 18s: until the 25th birthday, or the 26th birthday where the patient was 17 when treatment finished.
Storage	Capsules below 25C. Reconstituted suspension: refrigerate and discard after 7 days.

Information for the patient

Written information	Supply the flucloxacillin patient information leaflet.
Counselling	● Take on an empty stomach, an hour before food or two hours after, and finish the course. ● **Seek help THE SAME DAY if pain becomes severe or out of proportion, redness spreads quickly, the skin darkens or blisters, or you develop fever or feel generally unwell.** ● **Seek help if you see red streaks tracking away from the wound.** ● Stop and seek urgent help if you develop a rash, wheeze, or swelling of the lips or tongue. ● Report yellowing of the eyes or skin, or dark urine, even weeks after finishing. ● Keep the wound clean and dry. Come back if it is no better in 2 to 3 days.
Follow-up	As Arm 1.

Appendix 1 , Observations, red flags and tetanus

Every observation must be measured and recorded before any supply. Use the threshold for the patient's age band. If ANY threshold is breached, do not supply: refer. The bands match the Skin and Soft Tissue Infection PGD Appendix 1; the 12 and over row matches the Acute Bacterial Bronchitis PGD.

Temperature	• **REFER if 38C or above.**
Pulse	• **12 and over: REFER if above 110 at rest. 5 to 11 years: REFER if above 120. 2 to 4 years: REFER if above 140.**
Respiratory rate	• **12 and over: REFER if 22 or above. 5 to 11 years: REFER if 25 or above. 2 to 4 years: REFER if 40 or above.**
Blood pressure	• **12 and over: REFER if systolic below 100. UNDER 12: do not apply an adult threshold (a paediatric cuff and reference range are needed, and most pharmacies hold neither); use capillary refill, REFER if more than 2 seconds, and appearance.**
Oxygen saturation	• **REFER if SpO2 below 94% on air at rest.**
Level of consciousness	• **REFER on any new confusion or drowsiness; in a child, on any drowsiness, floppiness, or not responding normally to social cues.**

Necrotising fasciitis , emergency, not a referral

Any one of these means emergency assessment now.

- Pain out of proportion to the appearance of the wound.
- Rapidly advancing erythema, spreading over hours.
- Crepitus, or gas in the tissues.
- Skin necrosis, bullae, or dusky discoloration.
- Anaesthesia over the affected skin.
- Systemic illness out of keeping with the wound.

Other referral triggers

- Lymphangitis: red streaks tracking proximally from the wound.
- Spreading cellulitis, or erythema extending well beyond the wound margin.
- Deep or penetrating wound, retained foreign body, or a wound needing closure.
- Wound over a joint, tendon or bone.
- Bite to the face or hand.
- Immunosuppression, or diabetic foot wound.

Tetanus

- Establish and record the tetanus immunisation history for every patient.
- **A HIGH-RISK tetanus-prone wound (a tetanus-prone wound with heavy contamination by material likely to contain tetanus spores, such as soil or manure, and/or extensive devitalised tissue) needs immunoglobulin: refer the same day. Immunoglobulin is not covered by any GRH PGD. Where immunoglobulin and any vaccine dose have already been given for this wound and are recorded, tetanus management is complete and an infected bite or heavily contaminated wound may be treated under Arm 1 if otherwise eligible.**

- A tetanus-prone wound where the last tetanus-containing dose was more than 10 years ago, whatever the number of doses, or where the history is incomplete: a reinforcing dose is indicated (UKHSA Table 4) and may be given under the Tetanus (Td/IPV) PGD from age 10. Under 10, refer.
- Tetanus-prone wounds (UKHSA) include puncture wounds, wounds contaminated with soil or manure, wounds with devitalised tissue, burns, wounds with systemic sepsis, and wounds presenting more than 6 hours after injury.

Authorisation

Version	v011
Supersedes	Version 010, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

GET REAL HEALTH

Change history to version 004

Later versions are recorded in the Version and change record at the end of this document.

Version	v004
Date	9 September 2026
Changes	● The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. ● Paediatric flucloxacillin volume corrected. Version 002 read "Children 2 to 9 years: 250mg four times daily (10mL of the 250mg/5mL suspension)". 250mg of that suspension is 5 mL. The quantity box, 100mL for 5 days and 140mL for 7 days, was calculated correctly for 5 mL, so the parenthetical was the error and a child would have received double the intended dose. The identical error was found and corrected in the Skin and Soft Tissue Infection PGD on the same day; it had been copied between the two documents. Found by an automated internal consistency scan that recomputes every stated dose volume against its strength, not by anyone reading it. This is a targeted dose correction only: this document has not had a full clinical review and one remains outstanding. ● Later versions: see the Version and change record at the end of this document.

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
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Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....

Version and change record
Version: v011
Issued: 30 September 2026
Supersedes: Version 010, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v010
Issued: 14 September 2026
Supersedes: Version 009, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v009
Issued: 11 September 2026
Supersedes: Version 008, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Decision 29: Arm 1 (co-amoxiclav) applies only where tetanus management for the wound, including any HTIG, has been completed and recorded, or the wound is not tetanus-prone; otherwise refer. The Arm 1 introduction, indication, inclusion criterion, HTIG exclusion, tetanus action, records row and the Appendix 1 tetanus bullet state this, so a heavily contaminated wound whose HTIG was given at an emergency department can be treated.
Decision 31: the flucloxacillin arm now states the suspension dose and quantity for a child aged 10 to 17 who cannot swallow capsules: 500mg as 10 mL of the 250mg/5mL suspension four times daily, 200mL for 5 days or 280mL for 7 days.
Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.
Previous version records
Version: v008
Issued: 11 September 2026
Supersedes: Version 007, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
High-risk tetanus-prone wound is now defined as UKHSA defines it (a tetanus-prone wound with heavy contamination by material likely to contain tetanus spores and/or extensive devitalised tissue); presentation after 6 hours, puncture wounds and burns are listed as tetanus-prone, not high-risk, so the two tetanus lists no longer disagree. Burns and wounds with systemic sepsis remain outside the PGD.
Consent bullet in both arms now carries the house wording for under-16s (parental responsibility or Gillick competence, basis recorded).
Records retention now carries the 26th birthday rule for a patient aged 17 at completion.
Guidance summary now states that the flucloxacillin alternatives it describes are not authorised under this PGD.
Change history table now points to the version and change record for versions after 004.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no

dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v007

Issued: 11 September 2026

Supersedes: Version 006, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v005

Issued: 10 September 2026

Supersedes: Version 004, 9 September 2026

Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.

Version: v006

Issued: 10 September 2026

Supersedes: Version 005, 10 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

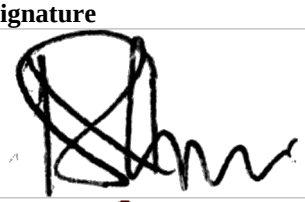
Changes in this version:

Appendix 1 observations are now by age band. Version 005 admitted children from 2 years in the flucloxacillin arm but carried a single set of adult thresholds, so a well 3 year old breached three of them. The bands match the Skin and Soft Tissue Infection PGD: 2 to 4, 5 to 11, and 12 and over, with capillary refill and behaviour in the paediatric bands and no adult blood pressure threshold under 12.

Tetanus aligned to the Tetanus PGD v007 and UKHSA Table 4: a tetanus-prone wound with the last dose more than 10 years ago gets a reinforcing dose under the Tetanus PGD whatever the dose count; an incomplete history alone is not a same-day referral; a high-risk wound needing immunoglobulin is.

Signed on behalf of Get Real Health

Version v011, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.